

UNIDERM 0.05% w/w Cream**TOPICAL CORTICOSTEROID**

Each 100 g contains:

Clobetasol propionate 0.05 g (0.05% w/w)● **DESCRIPTION:**

White, odorless, smooth, viscous cream

● **MECHANISM OF ACTION:****Pharmacology**

Clobetasol propionate is a highly active corticosteroid with topical anti-inflammatory activity. The major effect of Clobetasol propionate on skin is a non-specific anti-inflammatory response, partially due to vasoconstriction and decrease in collagen synthesis.

Pharmacokinetics

Percutaneous penetration of Clobetasol propionate varies among individuals and can be increased by the use of occlusive dressings, or when the skin is inflamed or diseased.

Mean peak plasma Clobetasol propionate concentrations of 0.63 ng/mL occurred in one study eight hours after the second application (13 hours after an initial application) of 30 g Clobetasol propionate 0.05% ointment to normal individuals with healthy skin. Following the application of a second dose of 30 g Clobetasol propionate cream 0.05% mean peak plasma concentrations were slightly higher than the ointment and occurred 10 hours after application. In a separate study, mean peak plasma concentrations of approximately 2.3 ng/mL and 4.6 ng/mL occurred respectively in patients with psoriasis and eczema three hours after a single application of 25 g Clobetasol propionate 0.05% ointment. Following percutaneous absorption of Clobetasol propionate, the drug probably follows the metabolic pathway of systemically administered corticosteroids, i.e. metabolized primarily by the liver and then excreted by the kidneys. However, systemic metabolism of Clobetasol has never been fully characterized or quantified.

● **INDICATION:**

Clobetasol propionate is a very active topical corticosteroid which is of particular value when used in short courses for the treatment of more resistant dermatoses such as psoriasis (excluding widespread plaque psoriasis), recalcitrant eczemas, lichen planus, discoid lupus erythematosus, and other skin conditions which do not respond satisfactorily to less active steroids.

● **DOSAGE AND ADMINISTRATION:**

Topical

Apply sparingly to the affected area once or twice daily until improvement occurs. As with other highly active topical steroid preparations, therapy should be discontinued when control is achieved. In the more responsive conditions this may be within a few days.

If no improvement is seen within two to four weeks, reassessment of the diagnosis, or referral, may be necessary.

Repeated short courses of UNIDERM Cream may be used to control exacerbations. If continuous steroid treatment is necessary, a less potent preparation should be used.

In very resistant lesions, especially where there is hyperkeratosis, the anti-inflammatory effect of UNIDERM Cream can be enhanced, if necessary, by occluding the treatment area with polythene film. Overnight occlusion only is usually adequate to bring about a satisfactory response. Thereafter improvement can usually be maintained by application without occlusion.

● **PRECAUTION:**

Long-term continuous therapy should be avoided where possible, particularly in infants and children, as adrenal suppression can occur even without occlusion. If UNIDERM Cream is required for use in children, it is recommended that the treatment should be reviewed weekly. It should be noted that the infant's napkin may act as an occlusive dressing.

If used in childhood or on the face, courses should be limited if possible to five days and occlusion should not be used.

The face, more than other areas of the body, may exhibit atrophic changes after prolonged treatment with potent topical corticosteroids. This must be borne in mind when treating such conditions as psoriasis, discoid lupus erythematosus and severe eczema.

If applied to the eyelids, care is needed to ensure that the preparation does not enter the eye, as glaucoma might result. If UNIDERM Cream does enter the eye, the affected eye should be bathed in copious amounts of water.

Topical steroids may be hazardous in psoriasis for a number of reasons including rebound relapses, development of tolerance, risk of generalized pustular psoriasis and development of local or systemic toxicity due to impaired barrier function of the skin. If used in psoriasis careful patient supervision is important.

Product	Code No.	Dimension	Packaging Type	Thickness
UNIDERM Cream	IUMY 0131	W 15 x L 26 cm.	Wood Free Paper (กระดาษปลอดกาว)	60 g (0.08 mm.)
 PM Specification Approved by: (PDM, ASPD, PDCM, PDC-A, PDCM, PDC-A) PLC: (Date)				
Designed by: (PDM, ASPD, PDCM, PDC-A, PDCM, PDC-A) Checked by: (PDM/ASPD)				
Approved by: (MD - Only Patient and Colant) QC-PM: (Date)				
CUSTOMER(SALES DEPT): (Date)				

Appropriate antimicrobial therapy should be used whenever treating inflammatory lesions which have become infected. Any spread of infection requires withdrawal of topical corticosteroid therapy and systemic administration of antimicrobial agents. Bacterial infection is encouraged by the warm, moist conditions induced by occlusive dressings, and so the skin should be cleansed before a fresh dressing is applied.

Use in pregnancy and lactation

There is inadequate evidence of safety in human pregnancy. Topical administration of corticosteroids to pregnant animals can cause abnormalities of fetal development including cleft palate and intrauterine growth retardation. The relevance of this finding to humans has not been established, therefore, topical steroids should not be used extensively in pregnancy, i.e. in large amounts or for prolonged periods.

The safe use of Clobetasol propionate during lactation has not been established.

● **SIDE EFFECT:**

Local hypersensitivity reactions such as erythema, rash, pruritus, urticaria, local skin burning and allergic contact dermatitis may occur at the site of application and may resemble symptoms of the condition under treatment.

As with other topical corticosteroids, prolonged use of large amounts, or treatment of extensive areas can result in sufficient systemic absorption to produce the features of hypercortisolism. This effect is more likely to occur in infants and children, and if occlusive dressings are used. In infants, the napkin may act as an occlusive dressing.

Provided the weekly dosage is less than 50 g in adults, any suppression of the HPA axis is likely to be transient with a rapid return to normal values once the short course of steroid therapy has ceased. The same applies to children given proportionate dosage.

Prolonged and intensive treatment with highly active corticosteroid preparations may cause local atrophic changes, such as thinning, striae, and dilatation of the superficial blood vessels, particularly when occlusive dressings are used or when skin folds are involved.

There are reports of pigmentation changes and hypertrichosis with topical steroids.

In rare instances, treatment of psoriasis with corticosteroids (or its withdrawal) is thought to have provoked the pustular form of the disease (see Special Warnings and Special Precautions for use).

If signs of hypersensitivity appear, application should be stopped immediately.

Exacerbation of underlying symptoms may occur.

● **CONTRAINDICATION:**

Rosacea; Acne vulgaris; Perioral dermatitis; Perianal and genital pruritus; Primary cutaneous viral infections (e.g. herpes simplex, chickenpox); Hypersensitivity to the preparation.

The use of Uniderm skin preparations is not indicated in the treatment of primary infected skin lesions caused by infection with fungi (e.g. candidiasis, tinea) or bacteria (e.g. impetigo); or dermatoses in children under one year of age, including dermatitis and napkin eruptions.

● **DRUG INTERACTION:**

None reported.

● **OVERDOSE AND TREATMENT:**

Acute overdosage is very unlikely to occur, however, in the case of chronic overdosage or misuse, the features of hypercortisolism may appear and in this situation topical steroids should be reduced or discontinued gradually, under medical supervision.

● **STORAGE:**

Store at temperature of not more than 30°C.

● **DOSAGE FORM AND PACKAGING AVAILABLE:**

Cream, Tube (Aluminum) 15 g

● **DATE OF REVISION:**

October 24, 2024

Manufactured by:
UNISON LABORATORIES CO., LTD.
39 Moo 4, Klong Udomcholjorn, Muang Chachoengsao,
Chachoengsao 24000 Thailand

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